

METROPOLITAN GENERAL HOSPITAL

2100 Healing Way, Boston, MA 02115 | (617) 555-0800 | www.metrogen.org

DISCHARGE SUMMARY

Patient Name: Marcus Lee	DOB: May 22, 1980 (Age 52)
MRN: MRN-8857296	Gender: Male
Admission Date: April 19, 2025	Discharge Date: May 08, 2025 (8 days)
Attending Physician: Dr. Angela Forsythe, MD — Cardiology	Insurance: Cigna #CIG-667-88901

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Community-Acquired Pneumonia	J18.9	Resolved
Iron Deficiency Anemia	D50.9	Resolved
Type 2 Diabetes Mellitus	E11.9	Chronic / Pre-existing

HOSPITAL COURSE

Marcus, a 52-year-old male with a known history of Type 2 Diabetes and Hypertension, presented to the Emergency Department on April 19, 2025 via ambulance with a 2-hour history of severe substernal chest pain radiating to the left arm, diaphoresis, and nausea. Initial ECG demonstrated ST-segment elevation in leads V1–V4 consistent with anterior STEMI. Troponin-I on arrival was critically elevated at 17.9 ng/mL. The patient was taken emergently to the cardiac catheterization laboratory where coronary angiography revealed 88% occlusion of the left anterior descending (LAD) artery. Successful percutaneous coronary intervention (PCI) was performed with deployment of a drug-eluting stent (Xience Skypoint 3.0 x 28mm). Post-procedural TIMI flow was grade 3. The patient was transferred to the Cardiac Care Unit (CCU) for monitoring. Serial echocardiography on day 2 showed an ejection fraction of 53%, mild anterior wall hypokinesia. Mild acute kidney injury (Creatinine 1.7 mg/dL) was noted, attributed to contrast nephropathy; this resolved with IV hydration by day 4. Blood glucose remained elevated throughout admission and endocrinology was consulted to optimize insulin regimen. The patient remained hemodynamically stable throughout and was discharged on day 8 in stable condition with close outpatient follow-up arranged.

VITALS AT DISCHARGE

BP	HR	SpO2	Temp	RR	Weight
133/68 mmHg	74 bpm	98% (RA)	97.8°F	18/min	85 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Aspirin	81 mg	Once daily	Indefinite
Clopidogrel (Plavix)	75 mg	Once daily	12 months
Atorvastatin	80 mg	Once at bedtime	Indefinite
Metoprolol Succinate	50 mg	Once daily	Indefinite
Lisinopril	10 mg	Once daily	Indefinite
Insulin Glargine (Lantus)	27 units	Once daily at bedtime	Ongoing — follow with Endo
Metformin	500 mg	Twice daily with meals	Ongoing

FOLLOW-UP PLAN

Cardiology: Dr. Forsythe — 1 week post-discharge. Repeat Echo at 6 weeks.
Endocrinology: Dr. Patel — 2 weeks post-discharge for diabetes management.
Primary Care: Dr. Kim — 4 weeks. Renal function panel to confirm AKI resolution.
Cardiac Rehab: Enrollment arranged — starts May 13, 2025 at MetroGen Rehab Center.

Labs: BMP, HbA1c, Lipid Panel — 2 weeks post-discharge.

Attending Physician Signature: _____

Dr. Angela Forsythe, MD — Cardiology | License #: MA-MD-41035 | Date: May 08, 2025

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